

1. Administrative & Study Identification

Full Study Title: A Phase 1/2, Multi-Center, Open-Label, Dose-Finding Study to Evaluate the Safety, Tolerability, Pharmacokinetics, Pharmacodynamics, and Preliminary Efficacy of BMS-986458, Alone and in Combination With Anti-lymphoma Agents in Participants With Relapsed/Refractory Non-Hodgkin Lymphomas (R/R NHL). **Short Title/Acronym:** A Study to Assess BMS-986458 Alone and in Combination With Anti-lymphoma Agents in Relapsed/Refractory Non-Hodgkin Lymphomas **Protocol Number:** 24-050 **NCT Number:** NCT06090539 **Posting ID:** 1011146812382055988 **Sponsor Name:** Bristol Myers Squibb **Trial Status:** Recruiting **Investigational Product:** BMS-986458 (a bifunctional cereblon-dependent ligand-directed degrader of BCL6) and Rituximab (Trade Names: Rituxan, Riabni, Ruxience, Truxima, Rituxan Hycela) **Phase of Development:** Phase I/II (PHASE1, PHASE2) **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead / Dr. Jennifer K. Lue

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, drug levels, and determine the optimal dose of BMS-986458 as a single agent and in combination with anti-lymphoma agents. **Secondary Objectives:** To evaluate preliminary biological and clinical activity, pharmacokinetics (Cmax, Tmax, AUC), and complete response rates of BMS-986458.

2.2 Background & Rationale To address the unmet medical need in patients with relapsed or refractory non-Hodgkin lymphoma (NHL), including DLBCL and Follicular Lymphoma, whose disease has returned or continues to grow after at least two prior lines of treatment. BCL6 helps lymphoma cells survive and grow; by breaking down the BCL6 protein, BMS-986458 provides a potential first-in-class, chemo-free therapeutic option to slow lymphoma growth and stimulate multi-modal anti-tumor efficacy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single Group Assignment (Dose-Finding) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: 308 participants **Number of Sites:** Approximately 35 locations globally **Estimated Study Start:** October 2023 **Estimated Primary Completion:** Up to 2 years and 1 month from study start

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants ≥ 18 years of age with R/R NHL (including DLBCL [de novo or transformed] and FL). 2. Following at least 2 prior lines of therapy (For de novo DLBCL/FL 3b: must have included combination chemotherapy with rituximab, an anthracycline, an alkylating agent, and steroids. For transformed DLBCL: lines administered after transformation. For FL except 3b: meeting treatment criteria). 3. Participant must have measurable disease defined by at least one FDG-avid lesion > 1.5 cm in the transverse diameter on cross-sectional imaging (CT or MRI). 4. Participants must accept and follow a strict pregnancy prevention plan.

4.2 Exclusion Criteria 1. Eastern Cooperative Oncology Group (ECOG) performance status ≥ 2 . 2. Prior CAR-T, or radiotherapy ≤ 4 weeks, systemic anticancer treatment ≤ 5 half-lives or 4 weeks, allogeneic SCT ≤ 6 months (for single agent or rituximab cohorts), or autologous SCT ≤ 3 months prior to study intervention initiation. 3. For BMS-986458 + T-cell engager cohorts: Prior alloSCT or solid organ transplantation; history of confirmed progressive multifocal leukoencephalopathy (PML); known/suspected hemophagocytic lymphohistiocytosis (HLH); known/suspected chronic active Epstein-Barr (EBV) infection. 4. Any condition, including significant acute/chronic medical illness, active/uncontrolled infection, or laboratory abnormalities placing the participant at unacceptable risk. 5. Inability to comply with listed restrictions, precautions, and prohibited treatments. 6. Known or suspected central nervous system involvement.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986458 administered orally (by mouth) at specified doses on specified days. Administered either alone or in combination with intravenous anti-lymphoma agents (Rituximab [ATC Code: L01FA01; Trade Names: Rituxan, Riabni, Ruxience, Truxima, Rituxan Hycela], Glofitamab/Obinutuzumab, or Mosunetuzumab). **Route of Administration:** Oral (BMS-986458) and Intravenous (Rituximab/combo drugs). **Duration of Treatment:** Up to 2 years and 1 month.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications. **Prohibited:** Concurrent systemic anticancer therapies, live virus vaccines, or concurrent participation in another therapeutic clinical trial.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Imaging (CT/MRI), Baseline Labs, ECOG
Baseline	Day 0	First Dose, Safety Assessment, Pregnancy Test
Interim	Cycles 1-X	Safety Labs, PK/PD Assessments (C _{max} , T _{max} up to 4 months), Imaging for Efficacy
End of Study	Up to 2 years & 1 month	Final Imaging, Exit Interview, Safety Follow-Up, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. Number of participants with adverse events (AEs). 2. Number of participants with serious adverse events (SAEs). 3. Number of participants with AEs meeting protocol-defined dose-limiting toxicity (DLT) criteria.

Measurement Method: Clinical observation, physical examinations, and laboratory evaluations graded per CTCAE criteria up to 2 years and 1 month.

7.2 Secondary & Exploratory Endpoints * Maximum concentration (C_{max}). * Time of maximum concentration (T_{max}). * Area under the plasma concentration-time curve within a dosing interval [AUC(TAU)]. * Number of participants with a complete response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Assessed continuously up to 2 years and 1 month to determine the safety and tolerability profile of the drug combination. **Serious Adverse Events (SAEs):** Evaluated continuously up to 2 years and 1 month; must be reported within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal dose-escalation safety review committee.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for preliminary efficacy; Safety Set for all participants receiving at least one dose of the study drug. **Sample Size Justification:** Target enrollment of N=308 appropriately powered to identify Maximum Tolerated Dose (MTD), recommended phase 2 dose, and detect preliminary biological/clinical signals. **Handling of Missing Data:** Last Observation Carried Forward (LOCF) or Multiple Imputation for continuous pharmacokinetic/pharmacodynamic endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring to ensure protocol adherence, dose-finding constraints, and strict SAE reporting.

Audit Trail: Electronic Case Report Form (eCRF) locking and comprehensive data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: {{24-050}} **Registry Number:** {{NCT06090539}} **Posting ID:** {{1011146812382055988}} **Sponsor/Lead Organization:** {{Bristol Myers Squibb}} **Trial Status:** {{Recruiting}} **Study Title:** {{A Study to Assess BMS-986458 Alone and in Combination With Anti-lymphoma Agents in Relapsed/Refractory Non-Hodgkin Lymphomas}} **Official Regulatory Title:** {{A Phase 1/2, Multi-Center, Open-Label, Dose-Finding Study to Evaluate the Safety, Tolerability, Pharmacokinetics, Pharmacodynamics, and Preliminary Efficacy of BMS-986458, Alone and in Combination With Anti-lymphoma Agents in Participants With Relapsed/Refractory Non-Hodgkin Lymphomas (R/R NHL).}}

12. Clinical Framework (Oncology/Lymphoma Specific)

Primary Condition / Disease Areas: {{Relapsed/Refractory Non-Hodgkin Lymphoma}} **Preferred UMLS & Medical Terminology:** {{Non-Hodgkin Lymphoma, Hodgkin Disease}} **Diagnosis Threshold:** {{Measurable disease > 1.5 cm FDG-avid lesion after >= 2 lines of prior therapy}} **Medical Methodology:** {{Targeted Protein Degradation / BCL6 Degradator Therapy}} **Optimization Target:** {{Optimal dose determination and tumor response}}

13. Study Procedures & Methodology

Management Pathway: {{BMS-986458 monotherapy or combination with Rituximab}} **Education Protocol (HCP):** {{Dose-escalation toxicity management protocol}} **Education Protocol (Patient):** {{Oral dosing compliance and strict pregnancy prevention plan adherence}} **Quality Audit Mechanism:** {{AE tracking & strict PK/PD sampling adherence}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 2 years and 1 month}} **Onsite Visit Cadence:** {{Cycles 1 through disease progression or end of study}} **Remote Monitoring Frequency:** {{Routine safety follow-up between

dosing cycles}} **Checklist Review Interval:** {{Prior to every
dosing cycle}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[03-Apr-2026]					
Clinical Operations Lead	[Name]	_____	[03-Apr-2026]					
Statistician	[Name]	_____	[03-Apr-2026]					